

FOR PEDIATRICIANS

When to refer a child who stutters.

Telling typical disfluency from early stuttering, in five slides.

Swipe →

WHY IT REACHES YOU FIRST

You are usually the first person asked.

Parents notice the repetitions and hesitations long before they think about speech therapy. The pediatric appointment is where the question gets raised, and where the next step gets decided.

THE DISTINCTION

Typical disfluency, or stuttering?

TYPICAL DISFLUENCY

- Occasional, brief repetitions
- No physical tension
- Ordinary pauses and fillers
- Clearer when tired, excited, or on new topics

SIGNS OF STUTTERING

- Repetitions, prolongations, or blocks, some over a second
- Visible physical tension
- Pauses that break a word apart
- Associated behaviours: blinking, foot tapping, rising pitch
- Present across most speaking situations

ALSO WORTH ASKING

How does the child react to it?

With typical disfluency, usually not at all. Where stuttering is emerging the reaction varies: some children seem untroubled, others show frustration, embarrassment, or a growing reluctance to speak. Both are worth writing down.

RISK FACTORS

Five things that raise the odds it persists.

- 01 Family history.** A close relative who stutters.
- 02 Later onset.** Starting after 3 years and 6 months.
- 03 Duration.** More than 6 months since it began.
- 04 Sex.** Boys are more likely to continue stuttering.
- 05 Language profile.** Advanced language for age, or a co-occurring speech or language difficulty.

THE REFERRAL

When in doubt, refer.

Signs of stuttering, or any risk factor present: refer for a full speech–therapy assessment.

Typical disfluency with worried parents: a referral still earns its place, because reassurance carries more weight when it follows an assessment.

AFTER THE REFERRAL

Therapy that keeps going between sessions.

UpSpeech is built with speech-language pathologists: AI-drafted session reports, and structured practice that carries on between appointments.

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